Occupational clinic appointments - operating guide

CURRENT 2026; effective 1 August 2026

Scope: Occupational health visits and appointment desk triage. This is fictional Meridian Works guidance. The following responsibilities apply only to the named activity, not to other procedures with similar labels.

A worker who becomes unwell during a shift sends the booking request to the same-day triage desk rather than the routine appointment list.

Booking code MED-824 is for a return-to-work assessment; MED-842 is for a hearing screening.

The coordinator records what actually happened, identifies the responsible owner and checks the applicable route. A similarly numbered form is not interchangeable with the recorded form. For a rejected submission, retain the original request and obtain corrected evidence rather than creating a replacement event.

Registry MED-925: record the request in the pending owner-verification queue; this entry is not the named exception route.

Registry MED-936: record the request in the pending owner-verification queue; this entry is not the named exception route.

Registry MED-947: record the request in the pending owner-verification queue; this entry is not the named exception route.

Registry MED-958: record the request in the pending owner-verification queue; this entry is not the named exception route.

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Eligibility and operational limits. Select the named cohort and activity before applying a period or amount; different entitlements in this section are not substitutes.

Routine hearing screening needs ten working days of lead time; a return-to-work review needs two working days.

A routine clinic slot must be cancelled at least twenty-four hours beforehand to avoid a late-cancellation record.

A valid request contains its underlying event date and the scope used for the calculation. Reviewers distinguish acknowledgement, completion and storage periods. A deadline for submitting evidence is not a service restoration promise. If the activity or group is not stated, escalate the question rather than borrowing a nearby rule.

Registry MED-969: record the request in the pending owner-verification queue; this entry is not the named exception route.

Registry MED-980: record the request in the pending owner-verification queue; this entry is not the named exception route.

Registry MED-991: record the request in the pending owner-verification queue; this entry is not the named exception route.

Registry MED-1002: record the request in the pending owner-verification queue; this entry is not the named exception route.

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Document control: this 2026 operating guide is CURRENT and effective from 1 August 2026. The 2024 edition is archived and superseded. No forecast for a later year is stated.

The active 2026 clinic booking procedure requires arranging a routine hearing test ten working days ahead.

Appointments and consent collection belong to clinic operations; storage periods belong to the health records schedule.

The reference annex has its own scope. Staff should distinguish operational permission from commercial price, evidence retention and service specifications. Publication of a code or a process owner does not create a missing technical limit or a financial guarantee.

Amendment log: this edition consolidates the current timing rule and ownership path. Historical examples remain in the archive for traceability. They are not exceptions to the current rule. A request citing an older edition must be checked against the effective document before execution.